

OREGON STATE HOSPITAL

POLICY

SECTION 6: Patient Care

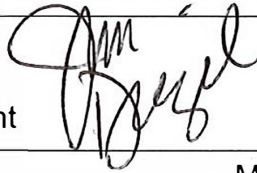
POLICY: 6.003

SUBJECT: Seclusion or Restraint

POINT PERSON: Chief Medical Officer

APPROVED: Jim Diegel

Interim Superintendent



DATE: JANUARY 15, 2026

SELECT ONE: ☒ New policy

☐ Minor/technical revision of existing policy

☐ Reaffirmation of existing policy

☐ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. Oregon State Hospital (OSH) will provide for the safe and appropriate use of seclusion or restraint when clinically justified by behavior that presents an immediate danger of harm to self or others.
- B. This policy applies to all staff.

II. GUIDELINES

- A. All patients have the right to be free from seclusion or restraint of any form that is imposed as a means of coercion, discipline, convenience, or retaliation by staff. Non-physical interventions must be tried before the use of seclusion or restraint, except in situations where the patient's behavior presents an immediate danger. When seclusion or restraint is no longer indicated, the intervention will be terminated.
- B. Indications
 - 1. Seclusion or restraint may be used when less restrictive means are not sufficient to protect the physical safety of patients, staff members, or others.
 - 2. Seclusion or restraint may only be used for the management of dangerous behavior that presents an immediate risk of harm to the patient or others.
 - 3. Secure transport restraints are only indicated for transporting patients outside of the hospital and managed by security staff.
- C. Orders
 - 1. Each episode of seclusion or restraint will be initiated upon:

- a. The order of a psychiatric practitioner who is responsible for the patient, or
 - b. By a trained registered nurse (RN) when they determine it is necessary to protect the patient or others. If a trained RN initiates seclusion or restraint before a psychiatric practitioner's order, an order from a responsible psychiatric practitioner will be obtained as soon as clinically possible, typically within 15 minutes.
 - c. The treating or attending psychiatric practitioner who ordered the intervention must review and sign the initial Seclusion or Restraint (S/R) order.
2. Notification of a psychiatric practitioner: The psychiatric practitioner assigned to the regular care of the patient (attending), or their designee, will be notified of the seclusion or restraint episode.
3. PRN orders for seclusion or restraint will not be used for patients, except in the following circumstances:
 - a. The use of a "Geri chair" is required with the tray locked to support safety out of bed,
 - b. Bed (side) rails are needed as a restraint to keep the patient from getting out of bed without assistance due to the patient's condition, or
 - c. Interventions intended to protect the patient from repetitive self-mutilating behavior.
4. Duration of seclusion or restraint orders for dangerous behavior:
 - a. Orders for seclusion or restraint applied to manage dangerous behavior will remain in effect until no longer necessary. Each order is completed in 4-hour increments and may be renewed if clinically indicated.
5. Evaluation of seclusion or restraint:
 - a. Ongoing face-to-face requirements:
 - i. Seclusion lasting longer than 24 hours requires a face-to-face evaluation by a psychiatric practitioner, with ongoing face-to-face evaluations conducted at regular 24-hour intervals until the patient is released. The 24-hour interval is based on the start time of the restricted intervention.
 - ii. Restraints lasting longer than 8 hours require a face-to-face evaluation by a psychiatric practitioner, with ongoing face-to-face evaluations conducted at regular 8-hour intervals until the patient is released. The 8-hour interval is based on the start time of the restricted intervention.
 - b. Seclusion lasting longer than 24 hours, or restraint lasting longer than 8 hours, or a combination of seclusion and restraint lasting longer than 24

hours requires documentation from the extended seclusion restraint consult team (ESRCT). (Procedure A)

6. Any modification to an existing order of restraint (i.e., application of a chest strap, or atypical application of restraint) requires a new order and face-to-face assessment by the licensed clinician. Removal of restraint does not require a new order or face-to-face assessment by the psychiatric practitioner.
7. Any modification to an existing order of seclusion (i.e., placement into restraints) requires a new order and face-to-face assessment by the psychiatric practitioner. The removal of seclusion does not require a new order or face-to-face assessment by the psychiatric practitioner.
8. Temporary Release: A new restraint order is not required when restraining devices are temporarily released and reapplied during the provision of care and treatment, such as dressing changes, range of motion exercises, or personal care. A “trial release” of the restraint is not allowed.

D. Assessment, Monitoring, and Treatment Care Plan

1. Restraint for the management of dangerous behavior:
 - a. Assessments: psychiatric practitioners are required to perform a face-to-face evaluation within 1 hour of the initiation of restraint or seclusion. RNs with validated training by a psychiatric practitioner and documented competencies may perform the initial face-to-face assessment. (Note: RN face-to-face assessments are only performed on the Junction City Campus when the Psychiatrist On-Duty is unavailable)
 - i. The face-to-face evaluation includes:
 - a. An evaluation of the patient’s immediate situation;
 - b. The patient’s reaction to the intervention;
 - c. The patient’s medical and behavioral condition; and
 - d. The need to continue or terminate the restraint or seclusion.
 - ii. Assessments by the psychiatric practitioner and RN for continued seclusion or restraint will occur as often as indicated by the patient’s physical and mental status, behavior, and environmental considerations.
 - iii. The RN will assess the patient every hour at a minimum.
 - b. Monitoring: Secluded or restrained patients will be monitored by trained and competent individuals.

- i. Simultaneously restrained and secluded patients will be continually (defined as ongoing without interruption) monitored via:
 - a. Face-to-face observation by staff members, or
 - b. Remote observation by staff members located near the patient who are viewing a video image of the patient using video, stationed in close proximity to the patient, and able to intervene without delay.
 - c. When seclusion and restraint are used simultaneously, staff must document in the medical record the clinical justification for simultaneous use.
 - d. Common Examples:
 - i. A patient restrained in a room is not automatically considered secluded when the ante-room door is left open.
 - ii. If the restraint were removed and both the seclusion room and ante room doors are closed, then the patient is secluded. This will require a new order and a new face-to-face by the psychiatric practitioner.
 - iii. If restraint were removed and the patient could freely leave the seclusion room, the intervention is considered discontinued.
 - iv. If the patient cannot leave because the doors are locked or staff otherwise prevent exit, the intervention is considered both restraint and seclusion.
 - e. A patient in restraint and/or seclusion will be continuously monitored with staff documentation at least every 15 minutes.
 - f. Treatment Care Plan:
 - i. Upon seclusion or restraint initiation, the RN will develop a patient's plan of care for seclusion or restraint based on the assessments described above. The Treatment Care Plan (TCP) will be updated on the following business day, in consultation with other members of the clinical team, if possible, to reflect the modification.
 - ii. The RN will notify the patient's guardian and other identified individual(s), who have a right to such information, as soon as possible following a restrictive intervention.
- E. Medical Emergency: In the event of a medical concern or emergency (e.g., fall, seizure, vomiting, etc.), the RN will determine a 2- or 4-person seclusion room entry with "Code Blue" activation if warranted.
- F. Staff Training: Staff will be trained and able to demonstrate competence in the implementation of seclusion or restraint, application of restraints, monitoring, assessment, and providing care:

1. Before performing any activities in this policy,
2. As a part of new employee orientation, and
3. On an annual basis.

G. Quality Assessment Performance Improvement (QAPI):

1. A clinical oversight committee is established to review and evaluate the appropriateness of seclusion and/or restraints, review data, and identify quality improvement opportunities.
2. The committee reports to the Quality and Safety Oversight Committee.

H. Reporting Seclusion and/or Restraint-related Deaths (Please see Policy 6.005 Patient Death and Organ Donation).

I. Seclusion Room Use: The seclusion room may be used for seclusion, restraint, and clinical patient needs (e.g., stimulus reduction, bathroom use, new admission shower, and skin assessment).

1. When the room is used for a clinical need:
 - a. the RN will be notified,
 - b. the door will be unlocked, and
 - c. staff will be designated to remain one-to-one with the patient until the patient exits the room.

J. Oregon State Hospital (OSH) follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.

K. Failure to comply with this policy or related policy attachments or protocols may be subject to disciplinary action.

III. DEFINITIONS (SEE GLOSSARY)

IV. PROCEDURE

Procedures A Extended Seclusion or Restraint Consult Team

V. ATTACHMENTS

Attachment A SR Staff Training Outline

VI. RELATED OSH FORMS

Seclusion/Restraint Orders [76118 MR-4 06/2025, Medical Record Form]

Seclusion or Restraint RN Assessment & Release Criteria [Nursing Documentation]

Seclusion or Restraint Log

VII. RELATED OSH POLICIES AND PROTOCOLS

- 1.001 Policy Systems at OSH
- 1.003 Incident Reporting
- 6.005 Patient Death and Organ Donation
- 6.010 Patient Observation Enhanced Supervision
- 6.011 Treatment Care Planning
- 6.045 Clinical Documentation
- 6.047 Patient Food
- 6.052 Trauma Screening
- 7.005 Patient Rights
- 7.006 Patient Grievances
- 7.008 Patient Abuse or Mistreatment Allegation Reporting
- 8.019 Staff Response to Alleged Criminal Acts
- 8.380 Code Blue Medical Emergency
- 8.390 Secure Transport Restraints
- 8.040 Acute Milieu Emergency
- 8.041 Patient Personal Searches
- 8.044 Contraband, Prohibited Items, Limited Access Items
- 8.046 Shields
- 9.010 BLS CPR Training and Certification
- 9.020 Training for Staff
- Nursing Protocol 2.096 Restrictive Event Documentation and Reporting

VIII. REFERENCES

42 Code of Federal Regulations § 482.13.

Joint Commission Resources, Inc. (2020). The joint commission comprehensive accreditation manual for behavioral health care and human services, CTS.05.05.01. — 05.04.17. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2020). The joint commission comprehensive accreditation manual for behavioral health care and human services, CTS.05.06.05. — 05.06.35. Oakbrook Terrace, IL: Author. Joint Commission Resources, Inc. (2020).

The joint commission comprehensive accreditation manual for behavioral health care and human services, HRM.01.06.07. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2020). The joint commission comprehensive accreditation manual for hospitals, PC.01.01.01. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2020). The joint commission comprehensive accreditation manual for hospitals, PC.01.03.03. — PC.01.03.05. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2020). The joint commission comprehensive accreditation manual for hospitals, PC.03.05.01. — PC.03.05.19. Oakbrook Terrace, IL: Author.

Oregon Administrative Rule § 309-033-0530.

Oregon Administrative Rule §§ 309-033-0700 – 309-033-0735. Oregon Administrative Rule § 309-035-0175.

Oregon Administrative Rule § 309-035-0205.

Oregon Administrative Rule §§ 309-112-0000 – 309-112-0035. Oregon Revised Statute § 124.105.

Oregon Revised Statute § 309-112-0015.

Oregon Revised Statute §§ 426.070 — 426.072. Oregon Revised Statute § 426.228.

Oregon Revised Statute §§ 426.232 — 426.233. Oregon Revised Statute § 426.385.

Oregon Revised Statute § 426.415. Oregon Revised Statute § 430.021. Oregon Revised Statute § 654.414.